

## Informed Consent

|                       |                             |                           |                                                                                                                              |
|-----------------------|-----------------------------|---------------------------|------------------------------------------------------------------------------------------------------------------------------|
| <b>Policy Number</b>  | POL-PC-066                  | <b>Owner / Department</b> | Medical Staff / Risk Management                                                                                              |
| <b>Effective Date</b> | 2023-03-01                  | <b>Version</b>            | 2.1                                                                                                                          |
| <b>Last Revised</b>   | 2026-01-22                  | <b>Next Review</b>        | 2028-01-22                                                                                                                   |
| <b>Approved By</b>    | Medical Executive Committee | <b>Applies To</b>         | All operative, invasive, and high-risk procedures and treatments requiring consent; all licensed providers and nursing staff |

### 1. Purpose

To ensure that patients (or their authorized representatives) make informed, voluntary decisions about proposed treatments and procedures through a documented informed-consent process, consistent with The Joint Commission informed-consent standards and CMS Conditions of Participation (42 CFR 482.13 and 482.51).

### 2. Scope

Applies to surgery, anesthesia, invasive procedures, blood transfusion, and other treatments for which the facility or law requires consent. Routine care covered by the general consent to treatment is excluded.

### 3. Definitions

- **Informed consent** — the process by which the provider performing the procedure discloses the nature, risks, benefits, and alternatives, and the patient voluntarily agrees.
- **Authorized representative / surrogate** — the person who consents when the patient lacks capacity or is a minor.
- **Emergency exception** — treatment provided without consent when delay to obtain it would seriously endanger the patient and consent cannot be obtained.

### 4. Required Elements of Disclosure

The provider who will perform the procedure is responsible for the informed-consent discussion. At a minimum, it covers:

- The patient's diagnosis and the nature and purpose of the proposed procedure or treatment.
- The material risks and expected benefits.
- Reasonable alternatives, including their risks and benefits, and the option of no treatment.
- Who will perform the procedure, and the likely result of declining.

### 5. Capacity, Surrogates, and Language Access

**IMPORTANT — Consent must be voluntary and obtained before the procedure and before sedation that impairs understanding. When the patient does not speak English or has a communication disability, use a qualified medical interpreter — never an ad hoc interpreter such as a family member — and document its use.**

- If the patient lacks capacity, obtain consent from the authorized surrogate per the state hierarchy; for a minor, from the parent or legal guardian except where law allows the minor to consent.
- Under the emergency exception, provide necessary care and document why consent could not be obtained.
- Confirm the signed consent is present, correct, and matches the planned procedure and site during pre-procedure verification (POL-SURG-007).

## 6. Severity & Reporting

Events are classified and reported under POL-RM-013. A procedure performed without valid informed consent, or not matching the consent (wrong procedure/site), is a SEV-1 sentinel-level event — notify the provider of record, Nursing Supervisor, Risk Management, and the Administrator-on-Call immediately and within 1 hour, and manage per POL-RM-004. A consent discrepancy caught during pre-procedure verification before the procedure begins is a near miss, reported within 24 hours. Resulting harm is disclosed under POL-PC-067.

## 7. Documentation Requirements

- Signed, dated, and timed consent naming the specific procedure, site/site, and provider.
- That risks, benefits, and alternatives were discussed and questions answered.
- Capacity determination and the surrogate's authority where applicable.
- Interpreter used (name/ID and method) when language access was needed.

## 8. References

- The Joint Commission, informed-consent standards (Record of Care / Rights).
- CMS Conditions of Participation, 42 CFR 482.13 (patients' rights) and 42 CFR 482.51 (surgical services — consent).
- Related: POL-SURG-007 (universal protocol), POL-PC-065 (AMA / informed refusal), POL-PC-067 (disclosure), POL-RM-013 (severity & reporting).